

reform that would dramatically increase the number of insured and therefore the number of prescription buyers, and improved profit margins from the continuing shift to generic drugs. CVS's private brands represented 17% of total sales. By contrast, Kroger, which operated grocery stores that included drugstores, had 28% private brand penetration. That, in turn, paled next to UK-based Tesco's 50%. This left CVS with a lot of runway for margin expansion. We expected the retail segment to continue to benefit from a greater volume of private label products that would offer a roughly 10 percentage point increase in gross margin.

Meanwhile, CVS had historic growth through acquisitions, and its management was beginning to focus its attention on the substantial potential for operational improvements. At one point, the retail and pharmacy division operated with seven inventory management systems, while Caremark had five different claims platforms. By 2013, their goal was to be down to one platform each, which should allow them to reduce retail store inventories by \$2 billion, representing about \$1.50 per CVS share, or 4.2% of the share price at that time. Moreover, we believed that CVS's Caremark PBM could improve its contribution to CVS EPS and potentially add an additional \$3–\$4 per share in value over three years.

We also liked that CVS's skilled management acted with the mentality of an owner/operator. They had substantial financial stakes in the company, which was likely to ensure they pursued what was in the best interests of shareholders.

We believed CVS's pharmacy and retail business had a larger moat than its PBM segment and offered a clear road map for sales growth and operational improvement. Given that there were publicly traded PBM peers like Medco Containment and Express Scripts, we believed we gained exposure to a business at a lower valuation by shorting